Too Many Pills?

Your Tools in One Place

From *Too Many Pills? A Nurse Practitioner's Guide to Medication Safety for Older Adults and the Families Who Help Them*

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How to Use These Pages

Every tool in *Too Many Pills? A Nurse Practitioner's Guide to Medication Safety for Older Adults and the Families Who Help Them* is gathered here, ready to print. The complete medicine list comes first, with the pages that keep it current, because every other tool builds on it. After that, the tools are grouped by what you're doing: noticing changes (Tool 4), getting ready for a visit or a review (6 to 8), changing a medicine (9 and 10), keeping the system running (11 and 12), and planning ahead (5, 13, and 14). Tool 15, a quick reference for your records, comes last. The chapter of the book where each one is explained is listed beside it, so you can go back to the reasons behind it.

Keep a notebook, a folder, or a note on your phone for each person whose medicines you keep track of. If you help two people, keep two. You don't need every page. Start with the complete medicine list and keep it in front, with a log of changes behind it. Whenever a medicine starts, stops, or changes, update both. Add other pages only when a problem or a change calls for them.

You may print or photocopy these pages for your own use or for someone you help. Keep your filled-in pages with your medicine list, and give a copy to whoever helps you.

One rule runs through all of them: these pages help you notice, ask, and plan. None of them is a reason to stop or change a prescription medicine on your own.

Contents

1 My complete medicine list 3
2 Change log 5
3 Questions for the whole list 6
4 Medicine notebook: what I've noticed 7
5 High-alert medicine safety plan 9
6 The question card 10
7 New medicine page 11
8 Brown-bag review checklist 12
9 My trial plan 13
10 Change tracker 14
11 Five links and the Medicine Year 15
12 Hospital stay: the three-list check 16
13 What Matters Now 17
14 When memory is changing 18
15 Your records: a quick reference 20

1 My complete medicine list

Chapters 3, 7, 8, 10, and 15

Name: _____ Date updated: _____

Keeps the whole picture (my captain): _____

My pharmacy (or pharmacies): _____

Allergies, and what actually happened: _____

The whole list on one page. Use one row per medicine. Put a star beside any blood thinner, insulin, diabetes pill that can cause low blood sugar, or opioid pain medicine, and give it a safety plan page (Tool 5).

Medicine and strength	What it's for	How and when I take it	Who prescribed it, and which pharmacy fills it	What it looks like

For a medicine that needs more room, copy this block:

Medicine and strength: _____

What it's for, in plain words: _____

How and when I take it: _____

Who prescribed it, or where I bought it: _____

Which pharmacy fills it: _____

What it looks like: _____

If I miss a dose: _____

List everything: prescriptions, medicines bought off the shelf, vitamins and supplements, creams, patches, eye and ear drops, inhalers, nasal sprays, and anything you take only sometimes. Write down the active ingredients, or bring the box. If no one can name what a medicine is for, put a question mark beside it. That's a question to ask, not a reason to stop it. Bottles usually show a medicine's own name; to see which group in the book it belongs to, look it up in "Matching Your Bottles to This Book," near the end of the book. Ask your pharmacist what to do if you miss a dose of each medicine, and write the answer on the last line of the block, or beside the medicine on the one-page list. When a medicine is stopped, cross it off with the date, and note the change in your change log (Tool 2).

2 Change log

Chapter 8

Date	What changed	Who changed it	Why, or questions to ask

When an after-visit summary and your list disagree, mark it as a question, not a mistake, and call before the next dose.

3 Questions for the whole list

Chapters 3, 7, and 9

The three questions

1. What is each medicine for?
2. Is that reason still true?
3. Is anything missing that should be there?

Then: Is there a simpler way? And the question families rarely think of: "Is there anything I should be taking that I'm not?"

The four cabinet questions

1. What do I take that didn't come from a prescription, including things I only use sometimes?
2. Is anything doing the same job twice, or hiding the same active ingredient twice?
3. Could any of it change how my prescriptions work, including what I eat and drink?
4. Does everyone who prescribes or dispenses my medicines have the same complete list?

Ask for a full medication review when:

- no one can say what one or more medicines are for
- medicines have been added by several different prescribers, or a new prescriber has joined your care
- there's been a hospital stay, a new diagnosis, or a big change in health since the list was last reviewed
- several medicines have changed in a short time
- you've been told your kidneys or liver are working differently
- there's a bottle in the cabinet you're not sure is still being taken
- you're unsure how to take your medicines

4 Medicine notebook: what I've noticed

Chapters 1, 2, 5, and 6

At the top, in your own words: *"This is different from how [he/she] was before. Could any of [his/her] medicines be contributing?"*

Date	What changed, in plain words	Roughly when it began	Any medicine started, stopped, or changed in dose in the past year, including anything bought without a prescription	Who noticed

Areas to watch: thinking and alertness, steadiness on the feet, eating, bowels and bladder, mood, and anything new on the skin.

What's changed about you? (Chapter 2) In the past year: weight lost or gained, an illness or infection, a hospital stay or a fall, eating or drinking less than you used to, becoming much more active or much less. Bring this page to your next medication conversation and ask whether any of these changes should change how a medicine is used. And ask once, on a calm day: "If I get sick and can't eat or drink normally, do any of my medicines need special instructions?"

Changes in thinking and balance (Chapter 6). Write down what you've noticed, in plain words, and roughly when it started. Begin with the look-backward questions: when did the fog or the falls begin, and what changed before they did? Then ask:

1. Could any of these medicines affect thinking, alertness, or balance?
2. Are several medicines adding to the same problem?
3. What else could explain what we're seeing?

Ask, too, whether anything on the list affects driving, and whether the time of day you take it matters. After a fall, mention eyesight, footwear, lighting, and the stairs.

Medicines started in the past year (Chapter 5). Fill this in for every medicine started in the past year.

Medicine	When was it started?	Why was it added?	What changed before it was added?

Look for a medicine added to treat a symptom that began after another medicine was started or changed. Look backward first:

1. When did it start?
2. What changed before it started?
3. Could something already on the list explain it?
4. Before adding something new, what do the clinician and pharmacist think?

When a new medicine is suggested for a new symptom, ask first: "Could any of the medicines already on the list be contributing?"

5 High-alert medicine safety plan

Chapter 4

Make one page for each blood thinner, each insulin or diabetes pill that can cause low blood sugar, and each opioid pain medicine. Get the answers from the prescriber or pharmacist, not by guessing.

Medicine, and what it's for:

What should we watch for?

What do we do if it happens, and how do we tell a phone call from an emergency?

How is this medicine monitored, and when is the next check?

What do we do when ordinary life doesn't go according to plan: a missed meal, an illness, a fall, a new medicine, or a week when the person who usually helps is away?

Who else knows this plan:

Keep it where others can find it, like the refrigerator, and tell at least one other person who sees you regularly.

6 The question card

Chapter 12

Front: for the prescriber

1. **Why this?** What does this medicine do, and why is it the recommended choice for me?
2. **Why now?** What changed that makes starting it right at this point?
3. **How will we know it's working?** What should change, and when will we check?
4. **What should I watch for?** What's common, and what means I should call?
5. **How long?** How long will I take it, and when will we look at it again?

Back: for the pharmacist, or if there's time

- How and when do I take it, and what if I miss a dose?
- Could this interact with anything I take, including over-the-counter medicines and supplements? Does it overlap with anything else I fill?
- Is there a simpler way?
- Is there a lower-cost option that would work as well?
- Could my symptom be caused by something I already take?
- Say it if it's true: "I've had trouble with a medicine like this before."
- Before I leave, is there anything about this medicine I should know?

Before you leave: "Let me make sure I understand what we're doing."

7 New medicine page

Chapter 12

Medicine:

For:

Check: (what should change, and when it will be checked)

Watch for: (what's common, and what means call)

Review: (when we'll look at it again)

8 Brown-bag review checklist

Chapter 9

Bring: prescription and over-the-counter medicines; herbal medicines, vitamins, and supplements; creams, liquids, inhalers, and anything you inject; pills in their original bottles, plus any organizers; things you take only sometimes; the mystery bottle; your complete medicine list; your medicine notebook, including the change log; recent after-visit summaries; your captain's name; your questions, written down.

Three doors: your pharmacist, your captain, or your Medicare drug plan. To the plan, ask: "Am I eligible for a comprehensive medication review?" Call the number on your plan card, not one from a letter or a caller.

Leave with: an updated, complete medicine list; a note of anything that changed, why, and who decided; any questions for another clinician, written down. Then send the updated list to your captain and your pharmacy, set aside anything you no longer need (see "Getting Rid of Medicines You No Longer Need" in the book), and put a note on the calendar for next year.

9 My trial plan

Chapter 13

A visit set aside for medicines is the time for the whole-list question: "Is there anything on this list I could stop or lower?" The three honest answers are **yes, no,** or **not yet.** If the answer is "not yet," ask: *"What would need to be true for us to reconsider this?"*

If you and your prescriber decide to try reducing or stopping a medicine, fill in this plan together before anything changes.

My trial plan	My answers
The medicine	
Why was it started, and is that reason still present?	
What are we trying to find out?	
All at once, or step by step?	
What should I expect, and what would concern my clinician?	
Who do I call if something changes?	
When will we check back?	
What would make us restart it?	

Keep the bottle until the trial is over, stored apart from your daily medicines, and restart it only if your clinician says so. The change tracker (Tool 10) carries you through the weeks that follow.

10 Change tracker

Chapter 14

At the top, from your plan:

What changed, and the date it changed: _____

What my clinician said I might notice, and for how long: _____

Call 911 now if: _____

Call today if: _____

Check-in date: _____

Date	What I noticed	What the plan said	What I did	What got better

Level	When	What to do
Keep watching	You were told it might happen, it's mild, and it's stable or improving	Write it down and keep an eye on it
Call today	It's unexpected, worsening, lasting longer than you were told, or you're unsure whether it fits the plan	Call your clinician's office that day (after hours, use the after-hours line)
Call 911 now	It could be dangerous: trouble breathing, chest pain, fainting, a seizure, signs of a stroke, sudden severe confusion or drowsiness, swelling of the face, lips, tongue, or throat, or bleeding that won't stop	Call 911 or get emergency care

Write down what got better, too, so your clinician can weigh it alongside any problems. With an antidepressant change, a noticeable drop in mood is always a reason to call that day. For thoughts of self-harm, call or text 988, or get emergency help. If someone helps you, give them a copy of the top half, so they know the plan too.

11 Five links and the Medicine Year

Chapter 15

Link	Where could it break?	Who holds it?
Store: one home for current medicines; stopped ones kept apart		
Sort: organizer filled from the current list, and checked		
Remind: each dose tied to something I already do		
Refill: synced refills at one pharmacy, or delivery		
Pay: cost said out loud if it's changing how I take anything		

My anchor month: _____

When	What to do	Where it's covered in the book
Every week	Fill or check the organizer against your current list	Chapter 15
Every refill cycle	Pick up synced refills; look ahead for holidays and trips	Chapter 15
Your anchor month	Update your medicine list	Chapter 7
Your anchor month	Do a brown-bag review with your pharmacist or clinician	Chapter 9
Your anchor month	Check that your records match reality	Chapter 10
Yearly	Medicare Annual Wellness Visit, including a review of your medicines	Chapter 13 and "Medicare Help With Your Medicines"
October 15 to December 7	Compare drug plans using your actual medicine list	Chapter 15 and "Paying for Your Medicines"

Go back to the system any time a medicine is added, stopped, or changed; after a hospital stay or emergency visit; when a cost changes; when the system starts slipping; or when the person who helps can't anymore.

12 Hospital stay: the three-list check

Chapter 16

The three handoff questions: What was I taking? What changed? What am I supposed to take now?

Label	What it means	What you need to know
Continue	Same medicine, same dose, same schedule	Nothing new; keep taking it
Stop	No longer part of the plan	Why it was stopped, and what to do with the bottle
Changed	Same medicine, different dose or schedule	The new instructions, and which bottle is current
New	Started during this stay	What it's for, and how long to take it
Hold	Paused for now, not stopped	When it restarts, and who decides

Before you leave, ask:

- For each medicine I took at home: Do I continue it, stop it, take it differently, or hold it?
- For each held medicine: Until when, and who decides when it starts again?
- For each new medicine: What is it for, and how long should I take it?
- Is anything on this list meant only for the time I was in the hospital?
- Who do I call this week if something doesn't make sense?
- When is my follow-up visit, and with whom?

At home, before the first dose: lay out your list from before the stay, the discharge list (and the rehab list, if there was one), and every bottle in the house. Give each medicine its label and match it to a bottle. Set stopped and held medicines aside in a separate bag, and keep held medicines until someone tells you whether they restart. If a medicine is stopped for good, see "Getting Rid of Medicines You No Longer Need" in the book. If two bottles might be the same medicine under different names, ask your pharmacist before taking either. If a high-alert medicine is new, start a safety plan page for it (Tool 5). If you can't label a medicine, call before you take it. If it's insulin, a seizure medicine, a Parkinson's medicine, or a transplant medicine, call the pharmacy or the after-hours line right away, even after hours, because those doses shouldn't be skipped. Never take everything on both lists to be safe.

Then call your captain the day you get home or the next business day. Say you've just been discharged, and ask for a follow-up visit.

13 What Matters Now

Chapter 17

Part 1: What matters most now. In the person's own words, before anyone mentions a medicine.

On a good day, what am I able to do?

What would I most hate to lose?

What am I willing to go through, and what am I not, to keep doing the things I care about?

Is there something coming up that I especially want to be there for?

Part 2: The job of each medicine.

- Does this help me feel or function better now, or is it meant to protect me later?
- If it's meant for later, about how long does it usually take to help?
- What does it cost me day to day: side effects, cost, extra pills, blood tests, risk of falls?

Part 3: The fit. Does this still serve what I said matters most? Take any doubts to your clinician.

Part 4: Who knows? Who else knows my answers to Part 1? _____

Where are they written down? _____

14 When memory is changing

Chapters 11 and 18

Steps of help, from least to most: share the list; set out the pills together; an organizer or pharmacy packaging; a reminder or locked dispenser; a helper gives each dose. Add only as much help as the person needs.

The question changes:

1. How much help do they need?
2. Can they still make this decision?
3. If they can't, what would they have chosen?
4. If no one can know, what is best for them now?

Hardest-time-of-day log

Date and time	What happened	What was going on just before	What helped

Bring the log to the next visit, with a note of who gives which medicines and any doses that were missed, refused, doubled, or found later. If a dose was doubled, call Poison Help (1-800-222-1222) right away, or 911 if the person is very drowsy, hard to wake, or having trouble breathing; don't wait for the visit. Bring your honest answer to one more question, too: how are *you* holding up?

When behavior changes, look for the cause first: pain; constipation or a full bladder; an infection or other new illness, including delirium; a medicine, especially a new one; poor sleep, hunger, thirst, or being too hot or cold; trouble seeing or hearing; depression, fear, or loneliness; too much noise, too little to do, or a change in routine; the time of day.

Four steps (DICE): *Describe* what happens, when, where, with whom, and what happened just before. *Investigate* the causes with the clinician. *Create* a plan together. *Evaluate*: keep what worked, and change what didn't.

Signs of pain someone can't put into words: breathing, sounds such as moaning or groaning, facial expression, body language such as guarding or rocking, and whether the person can be comforted.

If an antipsychotic is offered or prescribed, ask: What symptom is this meant to help, and how will we know if it does? What else have we checked, and what else could we try? What should

we watch for? When will we look at it again, and what's the plan for stopping it if it isn't clearly helping?

Also: ask whether a sleeping pill or anxiety medicine that's been started is still needed, and ask your pharmacist before crushing, splitting, or opening any pill. If Lewy body dementia has been diagnosed, make sure every clinician knows, including in an emergency room.

Speaking for them, ask: Did they ever say what they would want if they lost their memory, or what they wouldn't want? How did they feel about medicines, side effects, and being drowsy or "not themselves"? What did they care about most on an ordinary day? Which matters most to them now: comfort, calm, alertness, time at home? If you weren't there for those conversations, ask the people who were.

Before the next visit: find out who the health care proxy is and whether the office has it recorded, ask about a cognitive assessment and care planning visit, and call the Eldercare Locator to find your Area Agency on Aging (see "Where to Get Help" in the book).

15 Your records: a quick reference

Chapter 10

Your record is really several records: the medicine list in your clinic's record; visit notes; after-visit summaries; specialists' notes and lists; the pharmacy's fill history; and your own list.

Three questions for every medicine: What was prescribed? (your clinicians' records) What was dispensed? (your pharmacy's fill history) What are you actually taking? (at home)

Check your record

- **Medicines:** Is every medicine there, including ones from specialists? Is anything listed that you've stopped? Are doses and directions current?
- **Allergies:** Is every listed allergy something you actually reacted to? Is it described accurately, and is it an allergy or a side effect?
- **Recent visits:** Does the after-visit summary match what you were told? Does the visit note describe the conversation the way you remember it?
- **Your clinicians:** Are your current specialists listed? Is your captain listed as your primary clinician?
- **Your pharmacy:** Does your fill history match what you take? Is anything still being filled that a doctor stopped?
- **Your life at home:** Does the record match what actually happens in your kitchen every day?

When something is wrong: Start with a short message or a word at your next visit. If that doesn't fix it, or the error is in a visit note, you can formally ask the organization to amend the record. It generally has 60 days to respond, with one 30-day extension. If it says no, you can write a statement of disagreement that stays with the record.[1]

An allergy or a side effect? Tell your clinician what actually happened when you took the medicine: rash, trouble breathing, swelling, or something like an upset stomach. Ask whether the label should be clarified.

Family help: Use proxy access, a separate login in the helper's own name, rather than sharing your password. Ask what a proxy can see, whether you can limit it, and how to end it later. A health care power of attorney is something else; Chapter 11 of the book explains what it allows, and when.

Medicare: With a Medicare.gov account, you can download your claims data, including Part D drug claims, or share it with someone you trust.[2] Pharmacies will also print your fill history on request.

Read your record before each visit, and your visit notes a few days after.

Sources and Permissions

1. Amendment of protected health information. 45 CFR §164.526. Electronic Code of Federal Regulations. Accessed October 8, 2026. https://www.ecfr.gov/current/title-45/subtitle-A/subchapter-C/part-164/subpart-E/section-164.526
2. Medicare.gov. Share your Medicare claims. Centers for Medicare & Medicaid Services. Accessed October 8, 2026. https://www.medicare.gov/providers-services/claims-appeals-complaints/claims/share-your-medicare-claims

These pages offer general information about medicines and older adults. They are not medical advice for any one person and do not replace the care of a clinician or pharmacist who knows your history. Never stop or change a prescription medicine on your own because of something you read here.

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